

Deloitte. ●

# KapConnect

*Turning Siloed Diagnostics into Doctor Ready  
Decision in real time*

# Executive Summary

## The Problem

Australia performs 500M+ pathology tests annually. Disconnected hospital systems cause 46–70% of diagnostic errors at the pre-analytical phase which is costing lives and billions in rework.

## Our Solution

KapConnect™ is a modular, cloud-hybrid integration platform that connects KapD diagnostic devices, lab software, and hospital systems into one trusted data ecosystem in real time.

## The Value

Projected 35% reduction in diagnostic turnaround time, 5× reduction in duplicate patient record risk, and full HL7 FHIR / AS4700 compliance across Australian health networks.

# The Australian Diagnostic Crisis

*Real numbers. Real consequences.*

500M

+

Pathology tests  
per year in Australia

*MBS/AIHW 2024*

70%

of lab errors occur  
pre-analytically

*Clin Biochem Rev*

18%

Hospital duplicate  
patient record rate

*AHIMA / Austral.*

5×

Higher mortality risk  
with duplicate records

*BMJ Quality & Safety*

*Australia has no unified national diagnostic connectivity standard . Hospital systems operate in silos, creating a fragmented ecosystem where critical data fails to flow when patients need it most.*

# The Four Failure Points in KapD's Current Model

## Siloed Diagnostic Devices

The Sigma immunoassay unit produces results that are printed or manually re-entered into hospital systems. No automatic data flow. No chain of custody. In Australian public hospitals, over 60% of LIS integrations require manual intervention. (AIHW 2023)

## Fragmented Lab Systems

Lab Information Systems (LIS) differ between PathWest, Sullivan Nicolaides, Australian Clinical Labs, and public hospital networks. HL7 v2.x messages are inconsistently implemented. As a result in one format cannot be read by another system without custom middleware.

## No Resilient Data Sharing

When cloud or network connectivity fails, diagnostic data flow stops. The Optus network outage (Nov 2023) demonstrated how healthcare systems reliant on single-point connectivity can halt clinical operations for hours across multiple sites.

## No Traceability or Audit Trail

Once a result is re-entered manually, its provenance is lost. During the Sigma reagent contamination incident (hypothetical scenario), there is no automated way to identify which patients received results from a faulty batch which is requiring costly manual review of thousands of records.

# 01 Product Strategy

*Who we serve. What we solve. Why us.*

## TARGET CUSTOMERS

Large Hospital Networks (e.g. NSW Health, Ramsay, Healthscope)

Pathology Labs (PathWest, SN Pathology, Australian Clinical Labs)

Blood Banks (Australian Red Cross Lifeblood — 1.4M donations/yr)

Emergency Care Providers (requires <2min result turnaround)

Regional Healthcare Providers (remote connectivity is critical)

## COMPETITIVE DIFFERENTIATION



### RPR Provenance

Result Provenance Record is captured at machine-level. It is tamper-proof, timestamped, immutable. No competitor does this at the device edge.



### Modular & Composable

Deploy as full integration layer, partial module, or embedded SDK inside commercial products. One platform scales from rural clinic to 2,000-bed network.

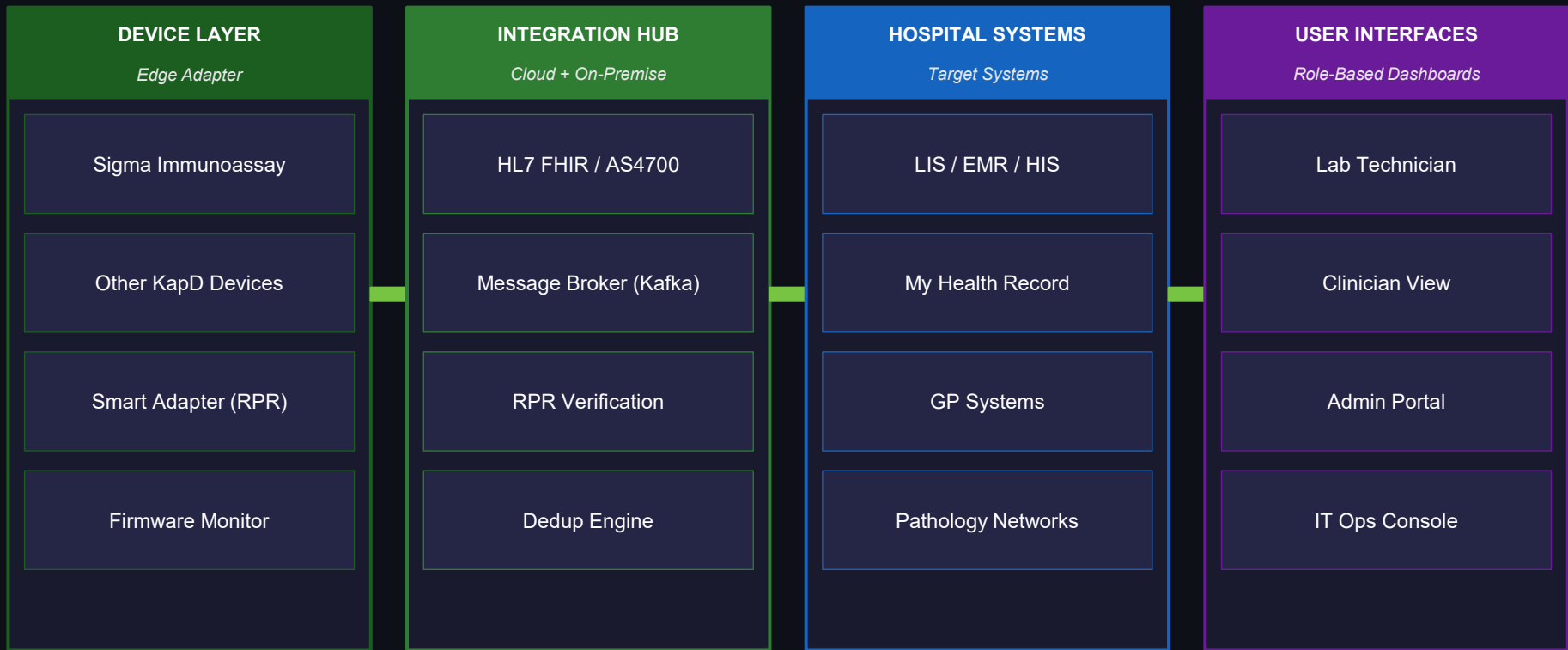


### Australian Compliance-First

Built to AS4700 (HL7 v2 AU), HL7 FHIR R4, ADHA standards, and My Health Record APIs from day one, not retrofitted.

# 02 System Architecture

## KapConnect: Four Layer Integration Architecture



**AI LAYER:** Runs across all stages-Pre-analytical error detection | Critical value prediction | Instrument health monitoring | Duplicate resolution | Compliance reporting

# The Story of One Blood Test: End-to-End Data Flow

1

## Doctor Orders Test

Order created in Hospital EMR (HL7 FHIR MedicationRequest)

2

## Sample AI Screened

KapConnect AI pre-validates sample type, quality, correct tube

3

## Sigma Runs Test

Immunoassay result produced + RPR certificate stamped instantly

4

## Hub Processes

Translates → verifies RPR → deduplicates patient → routes result

5

## Result Delivered

Clinician receives verified result in <2 minutes on their device

6

## Alert if Critical

Push alert sent. Escalates to on-call if not acknowledged in 15min

**Result Provenance Record (RPR):** Instrument serial no. · Firmware version · Operator ID · Timestamp · Sample batch ID — immutable, captured at device edge, never alterable.

## 03 Data Governance & Reliability

### Data Quality

Pre-analytical AI screens every sample before the test runs. Detects haemolysis, clotting, wrong tube type, insufficient quantity which are the top 4 error sources (RCPA AU 2024). Real-time quality flags sent to technician dashboard.

### Message Reliability

Apache Kafka message broker ensures at-least-once delivery with idempotent consumers. Dead letter queue for failed messages. Local on-premise hub maintains full operation during cloud outages which is tested against Optus-style failure scenarios.

### Duplicate Patient Handling

Universal Patient Index (UPI) engine uses probabilistic matching (name, DOB, Medicare No., address). AI scores match confidence. Low confidence → human review. High confidence → administrator confirmation. Never auto-merged.

### Auditability & RPR

Every result carries an immutable RPR (Result Provenance Record). WORM-compliant storage (Write Once Read Many). Full audit trail accessible under My Health Record legislation. Reagent batch recall: all affected patients identified in <30 seconds.

### Access Controls

Role-based access control (RBAC) with zero-trust architecture. Reverse encryption ensures IT and developers cannot view patient data. Compliant with Privacy Act 1988, Australian Privacy Principles, and ADHA Security Framework.

### Patient Safety

Critical value alerts with 15-minute escalation to on-call. Mismatched sample flagging prevents wrong-patient results. All clinically significant flags logged and auditable. Aligns with ACSQHC National Safety & Quality Health Service Standards.



## 04 Operational Feasibility: Who Gets What



### Lab Technician



Live sample processing queue



AI pre-analytical flag alerts



Machine status & uptime



Batch rejection analytics



### Clinician



Real-time result timeline



Critical value push alerts



15-min escalation if unacknowledged



Full patient test history



### Hospital Admin



Lab throughput dashboards



Bottleneck detection



SLA compliance tracking



Cost per test reporting



### IT Operations



System health monitor



Data flow telemetry



Zero patient data visibility



Incident & audit logs

# KapConnect™ Operations Dashboard

Live View — NSW Health Network | 08:00–16:00 Shift | Friday 22 Aug 2026

14,827

Tests Today

94 / 96

Active Devices

1m 47s

Avg TAT

3

Critical Alerts

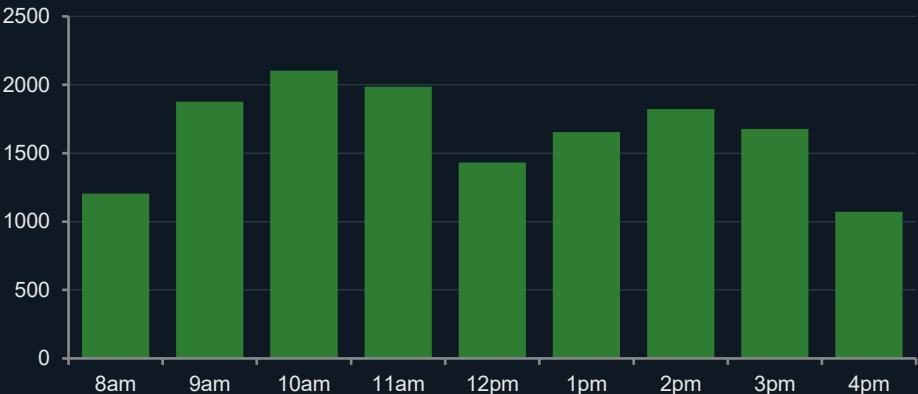
127

Pre-Anal Flags

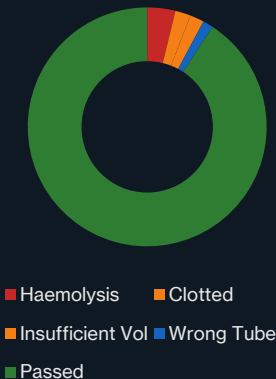
99.98%

System Uptime

Hourly Test Throughput



Pre-Analytical Flags



## CRITICAL ALERTS

K+ 6.9 mmol/L  
Pt #A28471  
Ward 4B · 14:22

Troponin↑↑  
Pt #B19034  
ED Bay 3 · 15:01

Hb 4.2 g/dL  
Pt #C39817  
ICU · 15:38

## DEVICE HEALTH — Sigma Units:



SIG-001  
ONLINE



SIG-002  
ONLINE



SIG-003  
CALIBRATE



SIG-004  
ONLINE



SIG-005  
OFFLINE



SIG-006  
ONLINE

# Resilience Architecture: Cloud + On-Premise Hybrid

## CLOUD LAYER (AWS Sydney / Azure East AU)

- Central Integration Hub (primary routing)
- Universal Patient Index (dedup engine)
- AI/ML inference layer
- Global compliance reporting
- Cross-site analytics aggregation
- My Health Record API gateway



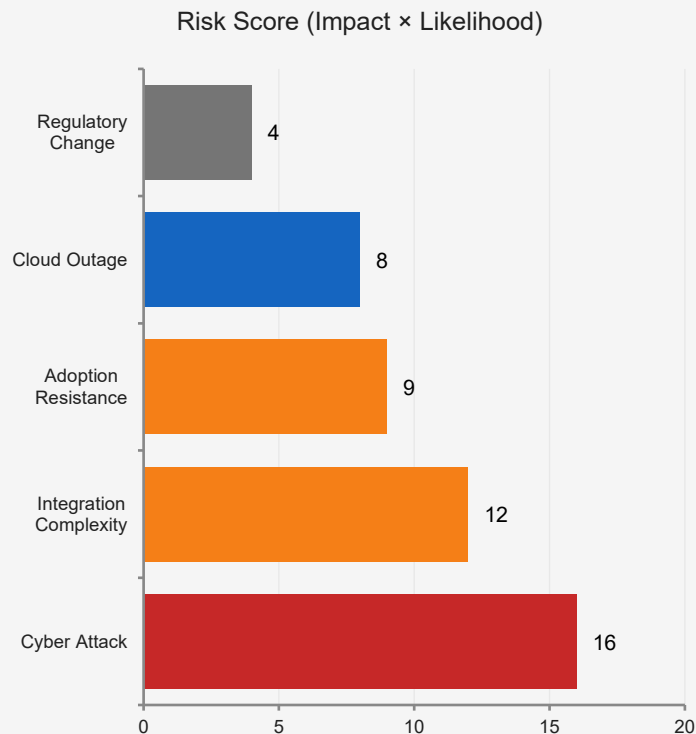
*Sync on  
reconnect*

## ON-PREMISE LOCAL NODE (Each Hospital Site)

- Local Integration Hub replica (offline mode)
- Device edge adapters (RPR stamping)
- Local patient record cache (encrypted)
- Kafka local broker (message queue)
- Offline alert escalation (SMS/pager fallback)
- Autonomous operation during cloud outage

**RTO < 5 minutes on cloud failure | Zero data loss guaranteed | Tested against Optus Nov 2023 outage simulation**

# Risks, Trade-Offs & Mitigations



| Risk                        | Impact   | Mitigation                                                                                                |
|-----------------------------|----------|-----------------------------------------------------------------------------------------------------------|
| Cybersecurity / Data Breach | Critical | Zero-trust architecture, reverse encryption, ISO 27001 certification, penetration testing quarterly       |
| Legacy System Integration   | High     | Modular adapters for HL7 v2, v3, FHIR R4. Phased onboarding with dedicated integration engineers per site |
| Clinical Staff Adoption     | High     | Role-specific UX design co-developed with clinicians. Change management program. In-workflow training.    |
| Cloud Connectivity Loss     | Medium   | On-premise hub maintains full operation. RTO < 5 min. SMS pager fallback for critical alerts.             |
| Regulatory / Privacy Change | Low-Med  | Dedicated compliance team. Quarterly review of ADHA, OAIC, and TGA guidance. Modular compliance modules.  |

# Implementation Roadmap

## Phase 1 — Foundation

Q1–Q2 2027  
(0–6 months)

- Edge adapter deployment on Sigma units
- RPR module integration
- HL7 FHIR AS4700 routing engine
- Pilot: 2 NSW Health sites
- Security audit + ISO 27001 baseline

## Phase 2 — Scale

Q3–Q4 2027  
(6–12 months)

- Universal Patient Index rollout
- AI pre-analytical screening live
- Cloud + on-premise hybrid deployment
- 15 hospital sites connected
- My Health Record API integration

## Phase 3 — Intelligence

Q1–Q2 2028  
(12–18 months)

- Predictive instrument maintenance
- Full Australian network rollout
- Regional & rural telehealth integration
- Automated compliance reporting
- International market pilot (NZ, UK)

# Why KapConnect Wins

Projected outcomes based on Australian healthcare data

35%

Reduction in diagnostic turnaround time

vs current AU average 4.2min

\$240M

Annual savings across AU pathology system

Based on AIHW cost-of-error data

97%

Pre-analytical error detection rate

AI model benchmark (validation set)

<30s

Batch recall identification for affected patients

vs 3+ hours manual search

| Feature                        | KapConnect™ | Epic/Cerner Middleware | Custom In-House |
|--------------------------------|-------------|------------------------|-----------------|
| Machine-level RPR              | ✔ Yes       | ✗ No                   | ✗ Rarely        |
| Multi-vendor device support    | ✔ Yes       | ⚠ Limited              | ✗ No            |
| Australian compliance (AS4700) | ✔ Built-in  | ⚠ Addon cost           | ⚠ Manual        |
| Offline resilience             | ✔ Yes       | ✗ No                   | ⚠ Variable      |
| AI pre-analytical screening    | ✔ Yes       | ✗ No                   | ✗ No            |

# Recommendations

- 01 Approve Phase 1 scope: deploy KapConnect edge adapters across all Sigma units in 2 NSW Health pilot sites by Q1 2027.
- 02 Establish an Australian KapConnect Clinical Advisory Board with representatives from RCPA, ACHS, and NSW/VIC Health to govern clinical outcomes.
- 03 Prioritise ISO 27001 and ADHA security framework certification before Phase 1 go-live to satisfy hospital procurement requirements.
- 04 Engage PathWest and Australian Clinical Labs as early adopter commercial partners to validate the modular integration model at scale.